



EVALUATION 2 — HIGH-PLUS ACUITY

Primary Diagnosis: Post-Surgical Bilateral Total Knee Arthroplasty with Wound Complication

PATIENT DEMOGRAPHICS

Field Detail

Patient Name Gerald "Jerry" Thomas Washington

Date of Birth 11/27/1952

Age 72

Sex Male

Gender Identity Male (he/him)

Race/Ethnicity Black / African American / Non-Hispanic

Primary Language English

Interpreter Needed No

Marital Status Married

Medical Record Number CI-2025-00423

Address of Service 3288 Redwood Terrace, Redwood City, CA 94063

Home Phone (650) 555-0837

Cell Phone (650) 555-0291

Email <jerry.washington52@email.com>

County San Mateo

EMERGENCY CONTACTS

Priority Name Relationship Phone (Primary) Phone (Alt) Lives with Patient?

1 Cynthia A. Washington Wife (650) 555-0838 (650) 555-0166 Yes

2 Marcus Washington Son (408) 555-0524 N/A No — San Jose

3 Denise Carter Sister (650) 555-0710 N/A No — Redwood City

LIVING SITUATION

Field Detail

Lives With Wife (Cynthia, age 70, generally healthy, active, primary caregiver)

Residence Type Single-story ranch home, owned

Stairs to Enter 1 step (no rail) to front door; accessible ramp installed at garage entrance

Interior Stairs None

Pets 1 small dog ("Duke"), Miniature Schnauzer, 12 lbs — active, may present tripping hazard

Home Access Instructions Garage keypad entry; code provided to agency by wife Cynthia

DME in Home Hospital bed (rented), bedside commode, elevated toilet seat with arms, rolling walker, front-wheeled walker (backup), CPM machine (bilateral — rented), ice therapy machine (GameReady), shower transfer bench

PAYER INFORMATION

Priority Payer ID / Policy Number Group Number Effective Date
Primary Medicare Part A (Traditional) 3DK7-RF2-WN44 N/A 12/01/2019
Secondary Blue Shield of California (PPO) — retiree benefit BSC-887412309 RET-5520 01/01/2015

PHYSICIAN INFORMATION

Role Name Specialty Phone Fax NPI
Attending / Certifying Dr. James P. Kowalski Orthopedic Surgery (650) 555-0944 (650) 555-0945 3456789012
PCP Dr. Evelyn Nakamura Family Medicine (650) 555-0400 (650) 555-0401 4567890123
Infectious Disease (consulting) Dr. Priya Mehta Infectious Disease (650) 555-1122 (650) 555-1123 5678901234
F2F Encounter Date: 06/14/2025 (inpatient, post-op day 2) F2F Performed By: Dr. James P. Kowalski

REFERRAL / HOSPITALIZATION HISTORY

Event Facility Dates Reason
Hospital Discharge Stanford Health Care 06/12/2025 – 06/17/2025 Bilateral simultaneous TKA; complicated by right knee surgical site dehiscence (1.5 cm) on post-op day 3
Pre-op clearance Dr. Nakamura office 05/28/2025 Cardiac and medical clearance for surgery
Start of Care Date: 06/18/2025 Referral Source: Stanford Health Care Ortho Discharge Team — Coordinator: Michael Chen, PT, (650) 555-1050

DIAGNOSES

Priority ICD-10 Code Diagnosis
Primary Z96.652 Presence of bilateral artificial knee joints (status post bilateral simultaneous TKA 06/12/2025)
Surgical M17.11, M17.12 Primary osteoarthritis, right knee; Primary osteoarthritis, left knee
Complication T81.31XA Disruption of surgical wound, not elsewhere classified, initial encounter (right knee dehiscence — 1.5 cm)
Complication T84.54XA Periprosthetic osteolysis — not currently applicable; monitoring for infection given wound issue
Secondary I10 Essential hypertension
Secondary E11.9 Type 2 diabetes mellitus without complications
Secondary E66.01 Morbid (severe) obesity due to excess calories (BMI 42.3)
Secondary G47.33 Obstructive sleep apnea (uses CPAP nightly)
Secondary M54.5 Low back pain (chronic, lumbar DDD)
Secondary K21.0 Gastroesophageal reflux disease with esophagitis
Secondary F17.210 Nicotine dependence, cigarettes, uncomplicated (smoking 1/2 pack/day x 40 years; told to quit pre-surgery, reduced to 3-4 cigarettes/day)

Surgical History

Bilateral simultaneous TKA (06/12/2025)
Right rotator cuff repair (2018)
Appendectomy (1989)
Left inguinal hernia repair (2001)

ALLERGIES

Allergen Reaction Severity

Codeine Severe nausea, vomiting Moderate

Adhesive tape Contact dermatitis, blistering Moderate

No other known allergies

CURRENT MEDICATIONS

Medication Dose Route Frequency Purpose High-Risk?

1 Oxycodone 5-10 mg PO Q4-6H PRN pain (max 40 mg/day) Post-op pain YES — OPIOID

2 Acetaminophen (Tylenol) 1000 mg PO Q8H scheduled (max 3000 mg/day with opioid) Pain, adjunct No

3 Gabapentin (Neurontin) 300 mg PO TID Neuropathic pain / surgical pain adjunct No

4 Enoxaparin (Lovenox) 40 mg SubQ Daily x 28 days post-op (through 07/10/2025) DVT prophylaxis YES — ANTICOAGULANT

5 Cephalexin (Keflex) 500 mg PO QID x 14 days (through 06/26/2025) Surgical wound infection prophylaxis (dehiscence) No

6 Metformin 1000 mg PO BID Type 2 DM Yes

7 Lisinopril 20 mg PO Daily HTN No

8 Amlodipine (Norvasc) 5 mg PO Daily HTN No

9 Omeprazole (Prilosec) 20 mg PO Daily (AM, 30 min before food) GERD No

10 Aspirin 81 mg PO Daily (HOLD during Lovenox per MD) Cardiac — currently held No

11 Ferrous sulfate 325 mg PO BID (post-op anemia — Hgb 9.2 at discharge) Anemia No

12 Stool softener (Docusate/Senna) 100 mg / 8.6 mg PO BID Constipation prophylaxis (opioid-induced) No

13 Nicotine patch 14 mg Transdermal Daily Smoking reduction/cessation No

Controlled Substances in Home: ☐ Yes — Oxycodone 5 mg tablets (qty 60, dispensed 06/17/2025) Sharps in Home: ☐ Yes — Lovenox prefilled syringes Sharps Container: Provided by pharmacy; in bedroom

CLINICAL ASSESSMENT — VITAL SIGNS AT ADMISSION

Parameter Value Abnormal?

BP 142/86 mmHg Mildly elevated

HR 76 bpm, regular WNL

RR 18 breaths/min WNL

Temp 99.1°F (oral) Borderline — monitor

SpO2 95% on RA WNL (low-normal for this patient)

Weight 312 lbs (141.5 kg) BMI 42.3 — morbid obesity

Height 6'1" (185.4 cm)

Pain 7/10 bilateral knees; 8/10 right knee (at wound site) Yes — expected post-op

Blood Glucose 204 mg/dL (2 hrs post-meal) Elevated — stress hyperglycemia + DM

CLINICAL ASSESSMENT — WOUND EVALUATION

Right Knee — Surgical Wound with Dehiscence:

Parameter Finding

Location Anterior right knee, midline surgical incision

Incision Length (total) 22 cm

Wound Status Closed with staples x 28 — EXCEPT mid-incision dehiscence

Dehiscence 1.5 cm length x 0.5 cm width x 0.3 cm depth, mid-incision

Wound Bed 80% granulation tissue (beefy red), 20% fibrinous (yellow, adherent)

Wound Edges Irregular, slightly macerated at inferior margin; staples intact above and below dehiscence

Drainage Moderate serosanguineous; 2x2 gauze saturated approximately 50% over 8 hours (per patient report)

Odor None

Peri wound Skin Erythema extends 1.5 cm circumferentially from wound edges; warm to touch; no induration, no fluctuance, no crepitus

Signs of Infection Peri wound erythema and warmth (monitoring closely); low-grade temp 99.1°F; on prophylactic antibiotics. No purulent drainage, no foul odor, no fever >100.4°F, no sepsis symptoms.

Current Dressing Aquacel Ag (antimicrobial) to wound bed; ABD pad secondary dressing; secured with rolled gauze and paper tape (NOT adhesive tape — allergy)

Dressing Change Frequency Daily and PRN if saturated

MD Wound Care Orders Cleanse with normal saline; apply Aquacel Ag to wound bed; ABD pad secondary; secure with rolled gauze and paper tape; change daily and PRN. If signs of infection worsen (expanding erythema, fever >100.4°F, purulent drainage), notify MD immediately.

Wound Photograph ☐ Taken with consent at admission

Staple Removal Per surgeon — follow-up appointment 06/26/2025 for staple assessment

Left Knee — Surgical Wound (Uncomplicated):

Parameter Finding

Location Anterior left knee, midline surgical incision

Incision Length 20 cm

Wound Status Closed with staples x 26 — all intact, well-approximated

Drainage Minimal serous; dressing with trace staining only

Peri wound Skin Mild ecchymosis (expected post-op), no erythema, no warmth

Signs of Infection None

Current Dressing Dry sterile gauze; paper tape

Dressing Change Frequency Every other day and PRN

HOMEBOUND STATUS ASSESSMENT

Criterion Finding

Criterion 1 MET. Patient is 5 days post bilateral TKA. Unable to ambulate more than 5-10 feet with front-wheeled walker and max assist of 1. Unable to navigate steps. Severe pain with weight-bearing (7-8/10). Morbid obesity compounds difficulty. Requires considerable and taxing effort to leave home — cannot get into a standard vehicle without significant assist.

Criterion 2 MET. Has not left home since discharge 06/17/2025. Only anticipated outings are surgeon follow-up appointments (medical care).

Homebound Status CONFIRMED

ADVANCE DIRECTIVES

Document Status

AHCD Yes — executed 2020. Wife Cynthia designated as health care agent.

POLST No

DNR No — Full Code

HOME SAFETY ASSESSMENT

Area Finding Recommendation

Entry (front) 1 step, no rail Use ramp at garage entrance exclusively until steps can be navigated

Entry (garage) Portable ramp installed; adequate width for walker; non-slip surface Adequate — monitor

Flooring Hardwood throughout; 1 area rug in living room (non-slip backing) Remove area rug (tripping hazard with walker)

Bathroom Shower transfer bench installed; handheld showerhead present; grab bar by toilet; elevated toilet seat with arms installed Adequate — verify stability of all equipment

Bedroom Hospital bed in place; bedside commode; clear path to bathroom Adequate

Lighting Good throughout None

Smoke/CO Detectors Present and functional (all 4 tested) None

Medications Oxycodone stored in bedroom nightstand (wife manages); Lovenox in refrigerator; all other meds in kitchen cabinet Secure controlled substance (recommend locking med box); ensure sharps container next to injection site

Oxygen Not applicable N/A

Electrical CPM machine and ice therapy machine require outlets near bed; extension cord in use Replace extension cord with heavy-duty power strip with surge protection; avoid overloading circuit

Dog (Duke) Active; runs to door, may get underfoot with walker Confine dog to separate room during patient transfers and ambulation; crate training recommended

Smoking Patient smokes 3-4 cigarettes/day on back patio (seated); using nicotine patch Reinforce smoking cessation; risk of wound healing impairment; risk of DVT; never smoke with nicotine patch on (toxicity risk); provide cessation resources

Emergency Access Address clearly marked; wide doorways accommodate walker Adequate

Overall Home Safety Risk Level: Moderate-High (primarily due to bilateral surgical wounds, opioid use, morbid obesity affecting safe transfers, and dog tripping hazard)

SERVICES ORDERED

Service Discipline Frequency Duration

Skilled Nursing RN Daily x 7 days (wound care), then 3x/week x 3 weeks, then 2x/week x 4 weeks 8 weeks

Physical Therapy PT Daily x 2 weeks, then 5x/week x 2 weeks, then 3x/week x 4 weeks 8 weeks

Occupational Therapy OT 3x/week x 4 weeks, then 2x/week x 4 weeks 8 weeks

Home Health Aide HHA Daily x 2 weeks, then 3x/week Duration of need

SKILLED NURSING CARE PLAN FOCUS AREAS

Wound Management

Right knee: daily wound assessment with measurement, photography, and documentation. Perform wound care per MD orders (NS cleanse, Aquacel Ag, ABD pad, paper tape). Assess for signs of infection (expanding erythema, fever, purulent drainage, wound culture if indicated).

Mark erythema borders with skin marker to track changes.

Left knee: assess every other day; dressing change per orders.

Staple line assessment bilaterally each visit.

Coordinate with surgeon re: staple removal timeline and wound closure plan.

Infectious disease consult follow-up — ensure cephalexin compliance and assess response.

DVT Prevention & Monitoring

Enoxaparin 40 mg SubQ daily — assess injection sites (rotate abdomen), teach patient/wife injection technique for days RN not present.

Assess for DVT signs each visit: unilateral calf swelling, warmth, tenderness, Homans' sign, new-onset dyspnea (PE concern).

Reinforce ankle pumps, early mobility per PT plan.

Monitor platelets (HIT risk with enoxaparin — lab draw per MD schedule).

Pain Management

Assess pain using numeric rating scale (0-10) at each visit — bilateral, separate scores for each knee.
Monitor opioid use: quantity tracking, assess for side effects (constipation, sedation, respiratory depression, urinary retention), educate on safe use.
Opioid tapering plan: coordinate with surgeon. Goal: transition from oxycodone to non-opioid analgesics within 3-4 weeks.
Assess for opioid misuse risk factors; use SOAPP or equivalent screening tool.
Non-pharmacologic pain management: ice therapy (GameReady), elevation, relaxation techniques, TENS if ordered.
Diabetes Monitoring

FSBS monitoring: BID (AM fasting + 2 hr post-meal) — anticipate stress hyperglycemia post-surgery.
Coordinate with PCP for potential insulin coverage or metformin dose adjustment.
Wound healing risk: educate on glycemic control's critical role in wound healing.
Smoking Cessation

Educate on impact of smoking on wound healing (vasoconstriction, hypoxia, increased infection risk) and DVT risk.
Nicotine patch compliance monitoring (reinforce: do NOT smoke while patch is on).
Motivational interviewing techniques; referral to California Smokers' Helpline (1-800-NO-BUTTS).
Post-Op Education

Teach signs of surgical site infection.
Teach signs of DVT/PE — seek emergency care immediately for sudden-onset dyspnea, chest pain, unilateral leg swelling.
Total knee precautions (per surgeon protocol).
Weight-bearing status and progression (per PT/surgeon).

PHYSICAL THERAPY FOCUS AREAS

ROM Progression — Bilateral knee flexion/extension; CPM machine use (settings per surgeon protocol — starting 0-90°, advance per tolerance)
Weight-Bearing Progression — Per surgeon: weight-bearing as tolerated (WBAT) bilateral with walker
Gait Training — Progressive ambulation with walker; transition to cane when appropriate; stairs when safe
Strengthening — Quad sets, straight leg raises, hamstring sets, ankle pumps → progressive closed chain exercises
Transfer Training — Bed mobility, sit-to-stand (high surfaces initially due to TKA precautions and BMI), car transfers (for MD appointments)
Balance — Static → dynamic balance progression; fall prevention
Bariatric Considerations — All interventions modified for 312 lb body weight; appropriate DME weight ratings verified; safe manual handling techniques

PLAN OF CARE GOALS (60-DAY CERTIFICATION PERIOD)

Goal Target

- 1 Right knee wound dehiscence fully closed with no infection 07/31/2025
- 2 Left knee incision fully healed, staples removed per surgeon 07/10/2025
- 3 Patient/wife demonstrate Lovenox injection technique independently 06/25/2025
- 4 Bilateral knee flexion $\geq 110^\circ$ 08/17/2025
- 5 Bilateral knee extension 0° 08/17/2025
- 6 Ambulate 300 feet with least restrictive device and no more than standby assist 08/17/2025
- 7 Transfer bed/chair/toilet independently 08/01/2025
- 8 Negotiate stairs (4 steps with rail) independently 08/17/2025
- 9 Pain controlled at $\leq 4/10$ with non-opioid regimen 08/01/2025
- 10 Zero DVT/PE events 08/17/2025
- 11 Fasting blood glucose consistently < 180 08/17/2025
- 12 Smoking reduced to 0 cigarettes/day 08/17/2025
- 13 Zero falls 08/17/2025
- 14 ADL independence (bathing, dressing, toileting) 08/17/2025

PATIENT SIGNATURES — EVALUATION 2

Patient Signature: _____ Date: _____

Print Name: Gerald Thomas Washington

Caregiver/Wife Signature: _____ Date: _____

Print Name: Cynthia A. Washington

Clinician Signature: _____ Date: _____

Patient / Representative Signature

Date

RN Clinician Signature

Date / Time